Lomustine for recurrent malignant glioma

Lomustine · 42 days NCCP 00805: recurrent malignant glioma. Use the exact prescription and schedule agreed with your specialist.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Single Day 1 lomustine dose

Active medicines: Lomustine

Emetogenic risk: Moderate to high (oral)

Medicine	Dose / route	When
Oral 5-HT3 antagonist - select ONE	Ondansetron 8-16 mg total daily OR granisetron 1-2 mg total daily; prescriber selects and records agent/ dose	At least 30 minutes before the single Day1 lomustine dose; record delayed-emesis cover and rescue duration NCCP V6 Table5 p8 and timing p4. Review QT risk, liver function, constipation and interactions. Lomustine is a single dose, not a daily course.

Additional phase-specific support

- Lomustine has moderate-to-high oral emetic potential. Take the prescribed prevention and rescue plan around the single dose. Contact the team if nausea or vomiting prevents fluids or essential medicines; do not automatically repeat a vomited chemotherapy dose.
- Keep prescribed steroids and seizure medicines on their own schedules. Ask about missed or vomited doses. Agree a bowel plan if other medicines cause constipation and check interactions before adding a medicine.
- Draft for clinician/pharmacist completion, not an issued prescription. Confirm agents, doses, timing, duration, interactions and rescue instructions with the current written plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00805 v2 · p3 · NCCP oral-systemic anticancer therapy (SACT) antiemetics V6: Table5 p8 and timing p4

Sources checked 2026-11-05 · SACTCheck v0.84.39 · Regimen-level drafts; prescriber and oncology-pharmacy verification required